

Study Report

“Qualitative Study on Social and Behavioural Determinants of Maternal, Newborn, Child, and Adolescent Well-being”



This report is prepared under the Social and Behaviour Change (SBC) Programme for Promoting Maternal, Child, and Adolescent Well-being, implemented by Sabalamby Unnayan Samity (SUS) with the support of UNICEF Bangladesh (2025), by A M Rasheduzzaman Khan

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EXECUTIVE SUMMARY

This report presents the findings from the Local-Level Qualitative Study on Social and Behavioural Determinants of Maternal, Newborn, Child, and Adolescent Well-being, implemented under the Social and Behaviour Change (SBC) Programme by Sabalamby Unnayan Samity (SUS) with technical and financial support from UNICEF Bangladesh. The study explored community-level behavioural drivers and barriers affecting health-seeking practices among women, caregivers, adolescents, and service providers across seven upazilas of Netrokona, Jamalpur, and Sherpur Districts between 14 and 27 October 2025.

The following sections present expanded thematic analyses (Chapter 3), gender and equity perspectives (Chapter 4), and conclusions with programmatic recommendations (Chapter 5).

The objectives were to understand (a) what behavioural and social determinants influence utilisation of ANC, PNC, EPI, and newborn care practices; (b) what beliefs and norms shape exclusive breastfeeding; (c) how communities perceive and act upon issues related to sexual exploitation, abuse, and accountability; and (d) how gender and equity dynamics determine access, decision-making, and feedback. A total of 20 Focus Group Discussions (FGDs) and 20 Key Informant Interviews (KIIs) were conducted with mothers of infants, pregnant and postpartum women, parents and caregivers, adolescents, influential community leaders, members of community groups (including Community Clinic Management Committees – CCMCs), and front-line health workers such as FWVs, CHCPs, and HAs including UHFPO. In total, 191 participants were engaged.

Thematic analysis was conducted manually through coding, categorisation, and triangulation across respondent groups. Data were interpreted to reveal the underlying behavioural patterns and contextual factors shaping service uptake and care practices.

Key Insights from Fieldwork: The key findings from the field study on maternal, newborn, child, and adolescent well-being are presented below. Based on data from Focus Group Discussions (FGDs), Key Informant Interviews (KIIs), and direct observations across seven upazilas in Netrokona, Jamalpur, and Sherpur, the findings highlight social and behavioural factors affecting health practices, service use, and protective behaviours. They reflect perspectives from mothers, caregivers, adolescents, community leaders, teachers, and health providers, showing both progress and ongoing challenges in knowledge, attitudes, and community norms.

Antenatal and Postnatal Care (ANC & PNC): Most women are aware that four ANC visits are recommended during pregnancy; however, in practice, many attend only one or two times. Barriers include distance to health facilities, limited transport, financial constraints, and household responsibilities. Attendance is often dependent on approval from husbands or mothers-in-law, and some women believe check-ups are unnecessary in the absence of complications. Dropout rates are particularly high at the fourth ANC visit, and PNC uptake is even lower, with only a small fraction of mothers completing all recommended visits. Targeted interventions should address household decision-making dynamics, engage male and elder family members,

provide transport support, and implement home-visit follow-ups to improve adherence.

Childhood Vaccination (EPI): While most mothers recognize the importance of vaccination, many lack knowledge of schedules and dose requirements. Fathers are generally less involved, leaving mothers and grandmothers responsible for ensuring timely vaccination. Children of families temporarily living away or with working parents often miss vaccines, and coverage gaps persist in remote areas. Awareness campaigns, male engagement, mobile outreach, and use of local influencers can help increase vaccination coverage and adherence.

Exclusive Breastfeeding (EBF): Mothers generally understand the importance of initiating breastfeeding within the first hour of birth and continuing exclusively for six months. Yet, practical challenges—such as delayed milk flow, infant illness, or traditional practices like giving honey or cow’s milk—limit adherence. Notably, educated or wealthier mothers are sometimes less consistent in exclusive breastfeeding. Interventions should emphasize lactation counseling, debunk harmful cultural beliefs, foster peer support groups, and involve male family members in promoting EBF.

Newborn Care: Families are mostly aware of essential newborn care practices, including giving breast milk within one hour, umbilical cord care, keeping the baby warm, and bathing after three days. Despite this, some poorer families neglect these practices, and certain harmful traditional practices persist. Effective interventions include household counseling on newborn care, practical demonstrations for cord care and warmth, follow-up visits by health workers, and engagement of grandmothers and elders as influencers.

Adolescent Health, Harmful Practices, and Service Utilisation: The qualitative findings from FGDs and KIIs in Kolmakanda (Netrokona), Bakshiganj (Jamalpur), and Sreebordi and Sherpur Sadar (Sherpur) reveal that adolescents continue to face interconnected challenges related to harmful social practices, limited access to adolescent-friendly health services, and persistent gender inequalities. Despite increasing awareness about the harms of child marriage, the practice remains common—driven by poverty, insecurity, social pressure, and entrenched norms that prioritize girls’ marriage over their education. While adolescents are familiar with national helplines such as 109, 1098, and 999, most rely on parents, teachers, or local leaders when facing violence or exploitation due to fear, stigma, and mistrust in formal mechanisms. Teachers reported efforts to raise awareness on child marriage, menstrual hygiene, vaccination, and personal health, but structural gaps persist, including the absence of school-based safeguarding and PSEA mechanisms. Adolescent girls were more informed about menstrual hygiene and TT vaccination, while boys were more exposed to child labour and peer violence. Across districts, socio-economic hardship, limited mobility, and gendered household roles restrict adolescents’ ability to access timely and confidential health and protection services. Participants emphasized the need for adolescent health corners, peer-support groups, and community-level awareness campaigns to promote rights, delay marriage, and ensure equitable access to services. Despite progress in knowledge

and attitudes, practical access to counselling, psychosocial support, and complaint mechanisms remains inadequate—highlighting the need for an integrated, gender-responsive, and behaviourally informed approach to adolescent well-being.

PSEA (Prevention of Sexual Exploitation and Abuse): Awareness of reporting channels is extremely low among most community members. Adolescents are somewhat aware and tend to report incidents to parents or teachers, yet fear, stigma, and lack of knowledge hinder broader reporting. Raising community awareness on PSEA, providing school-based sessions for adolescents, and establishing confidential reporting mechanisms are critical.

Service Gaps, Complaints, and Accountability: Most community members are unaware of where to report service deficiencies or lodge complaints. Existing feedback systems are largely ineffective, and communities often rely on informal reporting to local leaders. Strengthening complaint mechanisms, training health workers in grievance handling, and promoting transparency through community meetings and engagement of religious and community leaders can enhance accountability and service responsiveness.

Gender and Decision-Making: Women’s autonomy in healthcare decisions remains limited. Husbands and elder family members often make decisions regarding ANC, PNC, vaccination, and newborn care. Cultural norms restrict independent decision-making, though women’s awareness and knowledge are slowly increasing. Effective SBC interventions must include male engagement, couple counseling, peer support groups for women, and gender-sensitive messaging to promote shared decision-making and equitable access to health services.

Following table summarizing the issues, behavioral drivers/barriers, and suggested interventions:

Issue	Key Behavioral Drivers / Barriers	Suggested SBC Interventions
Antenatal Care (ANC) & Postnatal Care (PNC)	Awareness exists but most attend only 1–2 visits; high dropout by 4th visit; poor PNC follow-up; distance, cost, household duties, and need for husband/mother-in-law approval limit access; social norms treat pregnancy as a natural event not requiring frequent check-ups.	Targeted counselling for husbands and mothers-in-law; community transport support; mobile reminders and home visits to reinforce 4 ANC visits; community dialogue on importance of PNC and home visits within 48 hours of delivery.
Childhood Vaccination (EPI)	Mothers generally know vaccines but often forget schedules; fathers less engaged; children of working or migrant families miss doses; remote or hard-to-reach areas experience coverage gaps.	Strengthen awareness campaigns on full vaccination; involve fathers in child health sessions; mobile and outreach EPI sessions; engage religious and community leaders to

Issue	Key Behavioral Drivers / Barriers	Suggested SBC Interventions
		promote vaccination completion.
Exclusive Breastfeeding (EBF)	Most mothers know early initiation and 6-month exclusivity but face barriers such as delayed milk flow, maternal illness, household workload, and traditional feeding practices; some believe water or honey helps digestion.	Conduct mother-to-mother support group sessions; engage husbands and elders on EBF benefits; organize lactation counselling and demonstrations; address myths through community radio and courtyard sessions.
Essential Newborn Care (ENC)	Awareness of cord care, delayed bathing, and early breastfeeding exists but inconsistent; poorer families or those delivering at home skip hygienic practices; lack of understanding of thermal protection; few PNC visits.	Household demonstrations on newborn care; include ENC in SBC group meetings; promote health worker follow-up visits after home deliveries; engage grandmothers in ENC counselling.
Adolescent Health, Harmful Practices, and Protection	Early marriage, child labour, and gendered mobility restrictions persist due to poverty, social stigma, and traditional beliefs; low awareness of SRH rights and adolescent-friendly services; TT uptake good but PSEA and safeguarding mechanisms weak; boys face economic and peer pressure, girls face movement and safety barriers.	Introduce adolescent health corners and school-based sessions; organize peer support and life-skill education groups; engage parents and teachers on delaying marriage and supporting girls' education; promote awareness on reporting channels and psychosocial support; integrate youth voices in community campaigns.
Prevention of Sexual Exploitation and Abuse (PSEA)	Low awareness of PSEA; most report to parents, teachers, or local leaders rather than formal systems; fear of stigma and lack of trust in confidentiality deter reporting.	Integrate PSEA in school and community awareness programs; publicize helplines (109, 1098, 999); establish confidential complaint boxes in schools and clinics; build capacity of teachers and local leaders on PSEA response.
Service Gaps and Complaint Mechanisms	Communities unaware of where or how to file complaints; rely on informal networks; weak follow-up and limited accountability; lack	Establish visible, accessible complaint systems; build capacity of health staff on handling complaints;

Issue	Key Behavioral Drivers / Barriers	Suggested SBC Interventions
	of structured feedback mechanism.	promote social accountability meetings; disseminate hotline and contact info through SBC materials.
Community Feedback & Accountability	Community groups (CGs/CCMCs) exist but participation and follow-up are weak; limited sharing of outcomes; community members unsure if feedback leads to change.	Strengthen and regularize community meetings; engage local leaders and CGs in reviewing service performance; publicly display complaint resolution progress to build trust and transparency.
Gender & Decision-Making in Health-Seeking	Women's mobility and decision-making constrained by patriarchal norms; elders and husbands decide on ANC, PNC, or child immunisation; social restrictions affect adolescent girls more strongly.	Couple counselling; engage men and community influencers on shared decision-making; organize women's peer groups to boost confidence and agency; include gender equity messages in all SBC communication.

Conclusion: While awareness of maternal, newborn, and child health services has improved due to sustained SBC efforts, persistent social, cultural, and structural barriers continue to limit service utilization. Targeted, participatory interventions that engage women, men, elders, and community influencers are essential to promote behavioral change, improve service uptake, and ensure accountability in maternal and child health programs.



Chapter I: INTRODUCTION

This report presents the findings of the *“Local-Level Qualitative Study on Social and Behavioural Determinants of Maternal, Newborn, Child, and Adolescent Well-being”* conducted under the *Social and Behaviour Change (SBC) Programme for Promoting Maternal, Child, and Adolescent Well-being* implemented by Sabalamby Unnayan Samity (SUS) with technical and financial support from UNICEF Bangladesh. The SBC Project aims to address key social and behavioural barriers that limit the uptake of essential health and well-being services for mothers, newborns, children, and adolescents.

1.1 Research Background

The Social and Behaviour Change (SBC) Programme, implemented by Sabalamby Unnayan Samity (SUS) in collaboration with UNICEF Bangladesh, seeks to influence and transform social norms and behaviours that hinder access to and utilization of essential maternal, newborn, child, and adolescent health (MNCAH) services.

Despite steady national progress, many children and adolescents in Bangladesh—particularly those from vulnerable and remote communities—continue to face significant challenges related to survival, development, and protection. These include heightened risks of violence, exploitation, and abuse. Districts such as Netrokona, Jamalpur, and Sherpur are considered low-performing in several child rights and health indicators, including immunization coverage, maternal and newborn mortality rates, and the persistence of harmful practices like child marriage and violence against children.

According to the Bangladesh Demographic and Health Survey (BDHS) 2022, only 23.6% of women in the Mymensingh Division received at least four antenatal care (ANC) visits that included all recommended components. Essential newborn care practices have also declined between 2017 and 2022 among non-institutional births: appropriate cord care decreased from 47% to 41%, delayed bathing from 46% to 27%, and immediate breastfeeding from 69% to 40%.

Administrative data from the Civil Surgeon Office of Sherpur Sadar Upazila show that in 2024 alone, there were 12 reported maternal deaths and 53 neonatal deaths. Similarly, in Netrokona and Jamalpur districts, low immunization performance persists. For instance, in Kalmakanda Upazila, only 78.72% of children were fully immunized in 2024, while Jamalpur’s Melandoho and Islampur upazilas recorded similarly low coverage.

Bangladesh has reaffirmed its commitment to ending preventable maternal, newborn, and child deaths by 2030, aligned with the goals of Universal Health Coverage (UHC), the Sustainable Development Goals (SDGs), and the Every Newborn Action Plan (ENAP). To achieve SDG Target 3.2, the country must reduce under-five mortality by 44% and newborn deaths by 60%.

Complementing quantitative findings, the UNICEF (2023) Qualitative Study on Knowledge, Attitudes, Beliefs, and Practices (KABP) regarding child and adolescent care identified persistent gaps in community awareness about ANC checkups. Many caregivers do not consider ANC visits essential unless complications arise. Similarly, confusion remains among caregivers—particularly fathers—about the immunization